

Federalism Diagnostics: The Subsidiarity Layer for Brazil

Executive Briefing — A Governance Routing Protocol for SUS and Social Protection

The Problem: The Averaging Problem in SUS

Brazil's 1988 Constitution mandates universal, decentralized health care through SUS — the largest publicly funded health system in the world (220 million people). But federal allocation formulas cannot see local variation. A municipality in Maranhão (57% informality, 64% poverty) receives resources through the same formula as Santa Catarina (25% informality, poverty well below average). Informal care networks — community health agents, extended family eldercare, quilombola health practices — constitute significant health delivery in high-informality regions but are invisible to DATASUS and allocation systems.

The result: systematic mismatch between federal allocation and local need. Regional health disparities persist after nearly four decades of SUS. The Averaging Problem is also a racial health inequity problem: Black and pardo populations have systematically higher maternal mortality (2× the white rate nationally), and these disparities track the same informality and geographic patterns the allocation formulas cannot see.

The Solution: The Global Subsidiarity Index for SUS

Federalism Diagnostics introduces the **Global Subsidiarity Index (GSI)** — a measurement protocol that quantifies governance-tier mismatch in health delivery. It measures four dimensions (Decision Proximity, Knowledge Inclusion, Resilience Architecture, Cohesion) across sixteen indicators sourced from **IBGE, DATASUS, SIOPS, PMAQ, and PNAD Contínua** — Brazil's existing data infrastructure.

The GSI produces **routing recommendations** mapped to Brazil's governance structure: União → Estado → Região de Saúde → Município. When a policy is implemented at a different tier than the GSI recommends, the estimated cost is published as **Subsidiarity Drag** — making governance mismatch visible without overriding democratic decisions.

Three-Layer Architecture

Layer	Component	Function
1. Sensor	Civic Value Registry	LGPD-compliant registry making informal care capacity visible. Integrated into e-SUS AB as module. ACS workload adjustment funded. Voluntary. Local storage. No taxation linkage.
2. Diagnostic	GSI + Routing Matrix	16 indicators across 4 dimensions. BR-CAF complexity adjustment. Produces União/Estado/Região de Saúde/Município routing recommendations. Built on IBGE/DATASUS.
3. Integration	SUS allocation bridge	Connects GSI outputs to SUS transfer formulas, PMAQ quality metrics, and CIB coordination. Enables evidence-based allocation adjustment.

The Averaging Problem in Numbers

Indicator	Maranhão (NE)	Santa Catarina (S)	National
Informality rate	57.0%	24.9%	37.8%
Per capita income	~R\$1,319/mo (NE avg)	~R\$2,499/mo (S avg)	R\$2,020/mo
Poverty rate	63.8% (coastal)	Well below average	27.4%
Black/pardo maternal mortality	Above national disparity	Regional variation	2× white rate

Source: IBGE PNAD Contínua (2025); IBGE Summary of Social Indicators (2024); DATASUS maternal mortality data

The protocol is designed to be scientifically disprovable. If GSI-informed SUS allocation does not improve health delivery compared to existing formulas, the negative result still constitutes Brazil's first systematic municipal-level governance architecture diagnostic for health.

Implementation: How It Deploys

Phase 1: SUS Health Delivery (18–24 Months)

Element	Specification
Pilot scope	4–6 municipalities + matched controls. Selection criteria: at least one high-BR-CAF Nordeste, one low-BR-CAF Sul/Sudeste, one Amazônia, one state capital. Controls matched on population, BR-CAF, health region, and baseline ICSAP rates.
Partners	CEBRAP + FGV DAPP (academic) + Fiocruz (health expertise + evaluation) + IBGE/DATASUS (data) + CONASEMS (municipal) + SESAI (if Amazonian pilot)
Cost	R\$13–25 million over 18–24 months. Includes ACS workload adjustment and 25% contingency for institutional onboarding delays. Fundable via Fiocruz, FAPESP/FAPERJ, IDB, or PAHO.
Data launch tier	Silver (IBGE + DATASUS) from Day 1. Gold tier requires community health council integration during pilot.
Legal basis	1988 Constitution Article 198 (decentralization mandate). Law 8080/90 (SUS three-level governance). LGPD (data protection). No new legislation required.

Success Metrics

Metric	Baseline	Target
SUS allocation-need gap	Unmeasured	GSI-measured gap published for all pilot municipalities
ICSAP convergence	Divergent avoidable hospitalization rates	Statistically significant faster improvement in high-BR-CAF pilots vs. controls
Specialist referral wait	DATASUS baseline (Month 1)	≥10% reduction in pilots with GSI-informed CIB routing vs. control
Informal care visibility	0 (invisible)	Care capacity in e-SUS AB for pilot municipalities

Expansion Path

Phase 2 (Years 2–3): GSI applied across all municipalities in 2–3 pilot states. Domain expansion to education (FUNDEB) and social protection (Bolsa Família). CIB (Bipartite Committee) integration at state level.

Phase 3 (Years 4–5): National SUS allocation methodology integration. Results presented to IDB/ECLAC as replicable Latin American methodology. Academic anchor paper positions GSI as a federal governance diagnostic tool for the region.

What the Federal Government Gains

- **Improved targeting:** SUS transfers matched to measured local capacity, not administrative formulas
- **Visible outcomes:** Pilot vs. control municipalities provide evidence for what works
- **Reduced litigation:** Empirical diagnostics reduce STF allocation disputes
- **Racial equity visibility:** GSI makes structural health inequities measurable and targetable
- **Regional scaling:** Methodology replicable across Latin America via IDB/ECLAC

SUS made health a right. Federalism Diagnostics makes the delivery of that right measurable.

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